ATM SOFTEK
Digital X-ray, Pathology & ECG Center
Ambuja Eco Center, EM Block, Sec -V, Kolkata-700091

**MR. DEMO**

Age : 67Y
Sex : MALE
PID : DEMO / 2

**Sample Collected :** Self

**Referred By :** DR. RAJIB MD MEDICINE

Registered on : 26/07/2026
Collected : 26/07/2026
Reported : 26/07/2026

## HEMATOLOGY

| Investigation | Result | Unit | Reference Value |
|---|---|---|---|
| **Complete Blood Count ( CBC )** | | | |
| HB% ( Haemoglobin ) | 15.3 | gm/dl | 12 - 17 |
| RBC Count | 5.10 | million cells/mcL | 4.50 - 5.50 |
| WBC Count | 9800 | cu.mm | 4,000 - 11,000 |
| **DIFFERENTIAL COUNT** | | | |
| Neutrophils | 60 | % | 50 - 70 |
| Lymphocytes | 30 | % | 20 - 40 |
| Monocytes | 08 | % | 2 - 8 |
| Eosinophils | 02 | % | 1 - 6 |
| Basophils | 00 | % | 0.0 - 1 |
| **RED BLOOD CELL INDICES** | | | |
| PCV ( Hematocrit ) | 45.90 | % | 36.0 - 47.0 |
| RDW - CV | **36** | gm% | 11.9 - 14.8 |
| MCV | 90.00 | fl | 83 - 101 |
| MCH | 30.00 | pg | 27 - 32 |
| MCHC | 33.33 | gm/dl | 31.8 - 33.7 |
| PLT ( Platelet Count ) | 3.65 | Lakhs/cumm | 1.5 - 4.5 |
| PDW | 26 | % | 8.3 - 56.6 |

Note :

1. As per the recommendation of International council for Standardization in Hematology, the differential leucocyte counts are additionally being reported as absolute numbers of each cell in per unit volume of blood
2. Test conducted on EDTA whole blood.

Please Correlate With Clinical Condition

********** End Of Report **********



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## BIOCHEMISTRY

| Investigation | Result | Unit | Reference Value |
|---|---|---|---|
| **Liver Function Test** | | | |
| BILIRUBIN TOTAL | 0.36 | mg/dl | 0.1 - 1.0 |
| Bilirubin Direct<br>Methodology : Diazotized sulphanilic acid | 0.21 | mg/dl | 0.0 - 0.40 |
| Bilirubin Indirect<br>Methodology : Calculated. | 0.15 | mg/dl | < 1.10 |
| ALKP (Alkaline Phosphatase) | 125 | U/L | 42.00 - 141.00 |
| SGPT (ALT)<br>( Methodology : IFCC/UV-Kinetic Method ) | 32 | U/L | 5 - 40 |
| SGOT (AST)<br>( Methodology : IFCC/UV-Kinetic Method ) | 12 | U/L | 5 - 40 |
| Total Protein | 7.36 | | 6.0 - 8.5 |
| Serum Albumin | 4.21 | gm/dl | 3.2 - 5.5 |
| Serum Globulin<br>Methodology : Calculated. | 3.15 | g/dL | 2.0 - 3.50 |
| A:G RATIO<br>Methodology : Calculated. | 1.34 | | |

Note

1. In an asymptomatic patient, Non alcoholic fatty liver disease (NAFLD) is the most common cause of increased AST, ALT levels. NAFLD is considered as hepatic manifestation of metabolic syndrome.
2. In most type of liver disease, ALT activity is higher than that of AST; exception may be seen in Alcoholic Hepatitis, Hepatic Cirrhosis,and Liver neoplasia. In a patient with Chronic liver disease, AST:ALT ratio>1 is highly suggestive of advanced liver fibrosis.
3. In known cases of Chronic Liver disease due to Viral Hepatitis B & C, Alcoholic liver disease or NAFLD, Enhanced liver fibrosis (ELF) test may be used to evaluate liver fibrosis.
4. In a patient with Chronic Liver disease, AFP and Des-gamma carboxyprothrombin (DCP)/PIVKA II can be used to assess risk for development of Hepatocellular Carcinoma.

| Investigation | Result | Unit | Reference Value |
|---|---|---|---|
| **LIPID PROFILE** | | | |
| Serum Total Cholestrol<br>[CHOD-PAP ENZYMETIC METHOD,END POINT] | 169 | mg/dl | Diserable : < 200<br>Border Line : 200 - 240<br>High Risk : > 240 |
| Serum Triglycerides | 65 | mg/dL | 60 - 165 |
| HDL Cholestrol<br>[DIRECT CLERANCE METHOD] | **18** | mg/dl | <60 |
| LDL Cholestrol<br>[CALCULATED] | **138.00** | mg/dl | < 100 |

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| | | | |
|---|---|---|---|
| VLDL Cholestrol<br>[CALCULATED] | 13.00 | mg/dl | 2 - 30 |
| CHOL/HDL Ratio<br>[CALCULATED] | **9.39** | mg/dl | 3.3 - 4.4 |
| HDL Cholesterol<br>[DIRECT CLERANCE METHOD] | **7.67** | mg/dl | <60 |

Intrepretation : -
A pattern of lipids in the blood. A lipid profile usually includes the levels of total cholesterol, high-density lipoprotein (HDL) cholesterol, triglycerides, and the calculated low-density lipoprotein (LDL) cholesterol. This test is used to identify dyslipidemia (various disturbances of cholesterol and triglyceride levels), many forms of which are recognized risk factors for cardiovascular disease and rarely pancreatitis.

## Kidney Function Test

| | | | |
|---|---|---|---|
| Serum Urea | 35 | mg/dL | 10 - 50 |
| Serum Creatinine | 0.98 | mg/dL | 0.55-1.02 |
| Serum Uric Acid | 6.21 | mg/dL | 3.5 - 7.0 |
| SODIUM (Na+) | 142 | mmol/L | 135 - 145 |
| POTASSIUM (K+) | **3.2** | mmol/L | 3.5 - 5.0 |

Method : Automated Spectrophotometry Based Assay & Electrolyte Analyzer.

The kidneys play several vital roles in maintaining health. One of their most important jobs is to filter waste materials from the blood and expel them from the body as urine. They also produce and release erythropoietin (EPO), which stimulates the bone marrow to make red blood cells, renin, which helps control blood pressure, and calcitriol, the active form of vitamin D, which is needed to maintain calcium for teeth and bones and for normal chemical balance in the body. Among the important substances the kidneys help to regulate are sodium, potassium, chloride, bicarbonate, calcium, phosphorus, and magnesium. The right balance of these substances is critical. When the kidneys are not working properly, waste products and fluid can build up to dangerous levels in the blood, creating a life-threatening situation. Kidney function tests are common lab tests used to evaluate how well the kidneys are working.

Please Correlate With Clinical Condition
********** End Of Report **********